

Medication Administration Performance Improvement Program

IMPLEMENTATION PLAN

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Client:	Meridian Health and Wellness Network (Fictional)
ADDIE Phase:	Implementation — Phase 4 of 5
Program Cost:	\$33,360 total (blended learning program)
Go-Live Target:	Week 9 following project approval
Version:	V3.0 — July 2025

1. Purpose and Scope

This Implementation Plan describes the complete operational process for deploying the Medication Administration Performance Improvement Program at Meridian Health and Wellness Network. The program is a blended learning intervention addressing a medication administration error (MAE) rate of 11.2%, more than five times the industry benchmark of below 2%. The blended solution includes three scenario-based eLearning modules, simulation lab practice sessions, a weekly microlearning refresher series, and a laminated job aid at each medication preparation station.

This document covers all activities required to move the program from its completed development state to full operational delivery across three hospital facilities and two outpatient clinics. It addresses platform setup and LMS configuration, learner enrollment and communication, facilitator and supervisor preparation, pilot deployment, full rollout, and post-launch monitoring. It does not address content design or development, which are covered in the Design Document and documented in the associated case study narrative.

2. Program Overview

Program Element	Details
Total Participants	520 registered nurses across 3 hospital facilities and 2 outpatient clinics
Priority Cohort	Nurses with < 2 years tenure OR nurses who joined within 12 months of EHR deployment (n ≈ 140). These begin in Week 9. All remaining nurses begin in Week 10.
Delivery Components	3 eLearning modules (SCORM 1.2, self-paced) 1 simulation lab session per nurse Weekly 5-min microlearning series 1 laminated job aid per medication station
Platform	Meridian Health LMS (existing platform). eLearning hosted as SCORM 1.2 packages. Microlearning delivered via LMS mobile app.
Total Training Time	eLearning: 45 minutes (3 × 15 min modules) Simulation lab: 2 hours Microlearning: 5 min × 8 weeks ongoing
Completion Requirement	All three eLearning modules AND simulation lab session required for certificate of completion. Microlearning is ongoing reinforcement, not a completion gate.
Languages	English

3. Implementation Timeline

The implementation is structured across 12 weeks following project approval. The pilot phase (Weeks 7–8) is critical; findings from the Cohort 0 pilot directly inform revisions before full rollout begins.

Week(s)	Phase	Key Activities	Owner
1-2	Pre-Launch Prep	Finalize LMS configuration; upload and test SCORM packages on all 3 browsers and mobile; confirm enrollment lists with HR; draft and review all learner communications; print and laminate job aids (520 copies); set up reporting dashboards	ID + LMS Admin + IT
3-4	Facilitator Prep	Train simulation lab facilitators (4 facilitators, 8-hour training program); review simulation	ID + Education Manager

		scripts and equipment setup; brief Charge Nurses on their role in microlearning reinforcement; distribute facilitator guide and observation rubric	
5-6	Communication Launch	Send Tier 1 communication: email to all nurses (purpose, timeline, what to expect); brief Charge Nurses and Unit Managers in shift meeting; post LMS banner with enrollment information; answer FAQs via LMS announcement board	Education Manager + CNO
7-8	Pilot (Cohort 0)	Deploy to Cohort 0: 20 nurses representing all five facility units; collect real-time usage data from LMS; facilitate 2 simulation lab sessions; administer end-of-module surveys; convene pilot focus group in Week 8; incorporate findings into final LMS content and facilitator materials before full rollout	ID + Facilitators
9	Priority Cohort Launch	Full enrollment of ~140 priority nurses (< 2 years tenure); staggered simulation lab scheduling begins (Unit 1 and Unit 2 first); microlearning Week 1 auto-releases in LMS; Charge Nurses receive completion tracker for their unit	Education Manager
10-12	Full Rollout	All remaining nurses enrolled; simulation lab sessions continue (Units 3, 4, 5); job aids distributed to all medication preparation stations; weekly microlearning continues; completion rates monitored weekly; nurses falling below 50% completion by Week 12 contacted by Charge Nurse	LMS Admin + Charge Nurses
13+	Post-Launch Monitoring	Ongoing: microlearning series continues for 8 total weeks post-enrollment; completion reports sent to unit managers monthly; Kirkpatrick L3 survey at 90 days; MAE rate review by Quality and Patient Safety Department at 6 months	Quality + Education Manager

4. LMS Configuration and Technical Requirements

Item	Specification / Requirement
SCORM Version	SCORM 1.2 — compatible with Meridian Health LMS (tested prior to pilot launch)
Completion Tracking	Completion status: Passed/Failed based on quiz score ≥ 80%. Time-on-task tracked per module. Quiz data captured at item level for formative review.
Device Compatibility	Desktop: Chrome, Firefox, Edge (latest 2 versions). Mobile: iOS 14+ and Android 10+ via LMS mobile app. All modules tested on hospital-issued tablets before pilot.
Accessibility	WCAG 2.1 Level AA: closed captions on all audio, alt text on all images, keyboard navigable, high-contrast mode available.

Microlearning Delivery	Weeks 1–8 post-enrollment: one module auto-released each Monday morning. Push notification via LMS app. Estimated 5 minutes per session. Not tracked for completion — engagement data only.
Enrollment Method	Batch enrollment via HR data export (employee ID, unit, tenure date). Enrollment list reviewed by Education Manager before upload. LMS sends auto-enrollment confirmation email on day of enrollment.
Reporting	Weekly completion dashboards auto-generated in LMS. Distributed to Unit Managers and Charge Nurses every Monday. Escalation report (nurses not started by Week 10) sent to Education Manager.
Technical Support	LMS helpdesk: ext. 4-LEARN during business hours. Troubleshooting FAQ posted on LMS announcement board. Hospital IT escalation path documented for access/login issues.

5. Learner Communication Plan

Three communication touchpoints are planned before the first module releases, and two during delivery. All communications go through the Meridian Health internal communication channels (Workday notifications + email). Charge Nurses are the primary reinforcement channel at unit level.

#	Timing	Message	Message	Sender
1	Week 5	Program announcement: why it exists, what participation looks like, what learners will be able to do, how to access the training. Tone: patient safety framing, not compliance mandate.	Email + LMS	CNO
2	Week 6	Unit-level briefing: Charge Nurses receive a 10-minute brief script to share at the next shift meeting. Covers logistics, simulation lab scheduling, and the confidential reporting mechanism that launches alongside training.	Shift meeting	Charge Nurse
3	Enrollment day	Enrollment confirmation from LMS with direct link to Module 1. Includes: estimated time (45 min for all 3 modules), how to access on mobile, who to contact for technical help.	LMS auto-email	LMS system
4	Week 2 of rollout	Mid-point check-in: completion rate shared transparently ("40% of our nurses have completed Module 1"). Encouragement. Reminder of simulation lab sign-up link.	Email	Education Manager
5	Week 4 of rollout	Final completion reminder for any nurse who has not yet started. Personal outreach from Charge Nurse for nurses at 0% completion.	Email + direct	Charge Nurse

6. Simulation Lab Preparation and Scheduling

Component	Details
Facilitators	4 trained simulation lab facilitators. Each has completed the 8-hour facilitator training program and been assessed on the observation rubric before leading any simulation session.
Equipment	EHR test environment (sandbox — not connected to production data). Barcode scanners (one per workstation, confirmed functional before each session).

	Medication administration supplies (training-grade). Observation rubric and structured debrief guide.
Session Structure	2 hours per session. Groups of 6–8 nurses. First 20 minutes: orientation to EHR test environment. 60 minutes: guided practice scenarios (3 scenarios of increasing complexity). 40 minutes: structured debrief using observation rubric findings.
Scheduling	Sessions offered across all three shifts to accommodate rotating schedules. Nurses sign up via LMS calendar (max 8 per session). Minimum 2 sessions per week during rollout phase. Total sessions needed: approximately 70 (520 nurses ÷ 8 per session).
Completion Verification	Facilitator records attendance in LMS manually within 24 hours. Simulation completion automatically unlocks the module completion certificate for that nurse.

7. Job Aid and Performance Support Distribution

- Job aids are laminated (7" × 10") Quick-Reference Cards covering: the 5 Rights verification sequence in the EHR, key alert types with triage guidance, documentation sequence, and the incident reporting contact number.
- Quantity: 1 card per medication preparation station (estimated 65 stations across all facilities), plus 5 per unit as spares. Total print run: 350 laminated cards.
- Cards are printed by the Meridian Health print shop by Week 6, distributed by Charge Nurses at the unit-level briefing in Week 6.
- A digital PDF version is uploaded to the LMS and pinned to the program's resource page for easy mobile access during shifts.
- Cards are reviewed and reprinted annually or when EHR medication workflow is updated — whichever comes first.

8. Parallel Non-Training Interventions (Implementation Coordination)

Three non-training interventions were identified during the Front-End Analysis as required alongside the training program. Implementation coordination for these interventions is tracked below.

Intervention	Implementation Action	Owner & Timing
Anonymous incident reporting mechanism	IT to configure anonymous reporting form in Workday; QA team to create non-punitive reporting policy one-pager; CNO to communicate policy change in all-staff email before training launches	IT + QA + CNO — complete by Week 5
EHR alert tiering review	IT and Pharmacy joint review of all EHR alert categories; categorize as Critical / Informational / Administrative; configure alert display to surface Critical alerts more prominently; reduce non-actionable alert volume by target 40%	IT + Pharmacy — complete by Week 8 (pilot phase)
Barcode scanner maintenance — Units 3 and 5	Facilities team to assess all scanners on Units 3 and 5; replace non-functional units before pilot begins; establish monthly maintenance schedule going forward	Facilities — complete by Week 6

9. Post-Launch Evaluation and Monitoring

Level	Metric	Collection Method	Target
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L1 — Reaction	Module satisfaction; perceived relevance; simulation usefulness	End-of-module survey (3 items, 5-point Likert + open comment); post-simulation survey (5 items)	≥ 4.2 / 5.0
L2 — Learning	Module quiz scores; simulation observation rubric scores	LMS quiz data (auto-captured); Facilitator observation form from each simulation session	≥ 80% quiz pass rate; rubric ≥ 3/4 on all criteria
L3 — Behavior	On-the-job protocol compliance; near-miss reporting rate	90-day structured survey to all completers; Charge Nurse observation log; LMS barcode scan completion data for 6 months	≥ 70% report applying 5 Rights protocol without prompting within 90 days
L4 — Results	MAE rate; near-miss report volume; documentation accuracy	Quality and Patient Safety Department quarterly incident review; EHR documentation error rate report	MAE rate ≤ 4% within 6 months; ≤ 2% within 12 months

10. Go-Live Readiness Checklist

All items below must be confirmed complete before full rollout launches in Week 9. The Education Manager is responsible for obtaining sign-off from each owner.

Readiness Item	Owner	Status
All 3 eLearning modules uploaded and tested in LMS (desktop + mobile)	LMS Admin	
SCORM tracking confirmed: completion status, quiz scores, time-on-task captured correctly	LMS Admin	
Microlearning Week 1 scheduled in LMS for auto-release at enrollment	LMS Admin	
Enrollment lists validated with HR and uploaded		
Enrollment confirmation email tested and confirmed sending correctly	LMS Admin	
All 4 simulation facilitators trained and assessed on observation rubric	ID / Education Manager	
EHR sandbox environment confirmed functional (not connected to live data)	IT	
Simulation lab calendar open for booking in LMS; all shift slots available	LMS Admin	
All 350 job aid cards printed, laminated, and delivered to unit Charge Nurses	Facilities	
PDF version of job aid uploaded to LMS resource page	LMS Admin	
Anonymous reporting mechanism live in Workday; non-punitive policy communicated	QA + IT + CNO	
EHR alert tiering review complete; critical alert display updated	IT + Pharmacy	
Barcode scanners on Units 3 and 5 assessed and replaced as needed	Facilities	
All-staff program announcement sent (Comm #1)	CNO	

Unit-level Charge Nurse briefings completed (Comm #2)	Charge Nurses	
Weekly completion dashboard confirmed generating correctly in LMS	LMS Admin	
Pilot (Cohort 0) evaluation data reviewed; critical revisions incorporated	ID	